

THESIS SAMPLE • SUBJECT COURSE: NURSING

**COMPASSION FATIGUE, BURNOUT, AND
TURNOVER INTENTION AMONG
REGISTERED NURSES IN SELECTED
TERTIARY HOSPITALS IN DAVAO CITY,
PHILIPPINES**

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CHAPTER 1

THE PROBLEM AND ITS BACKGROUND

Introduction

The nursing profession occupies an indispensable position within the Philippine healthcare system, serving as the primary point of contact between patients and the institutional mechanisms of care across hospitals, community health centers, and specialty facilities nationwide. Registered nurses constitute the largest professional group in Philippine healthcare, numbering approximately 600,000 active practitioners as of 2023, according to the Professional Regulation Commission (PRC, 2023). Their clinical competence, emotional availability, and professional commitment are foundational determinants of patient safety, care quality, and health system performance at every level of the national healthcare hierarchy.

The demands placed upon Filipino registered nurses have intensified significantly in the decade preceding this study, reaching a critical apex during and following the COVID-19 pandemic of 2020 to 2022. The pandemic required nurses to sustain prolonged direct contact with severely ill patients under conditions of acute resource scarcity, inadequate personal protective equipment, organizational understaffing, and profound personal risk of infection and death. The psychological toll of these conditions on the nursing workforce has been extensively documented internationally, with studies consistently identifying alarming

elevations in compassion fatigue, clinical burnout, and voluntary turnover intention among nurses who served during the pandemic period (Labrague and De Los Santos, 2020; Hofmeyer et al., 2020).

Compassion fatigue, defined as the emotional, physical, and spiritual exhaustion that can occur when nurses take on the suffering of their patients without adequate recovery, represents a progressive occupational hazard that disproportionately affects nursing professionals relative to other healthcare workers due to the intimate and sustained nature of nurse-patient relationships (Figley, 1995). Left unaddressed, compassion fatigue progresses toward clinical burnout, a state of chronic emotional exhaustion, depersonalization, and diminished sense of personal accomplishment that fundamentally impairs a nurse's capacity to provide compassionate, high-quality care (Maslach et al., 2001). Both conditions are strongly associated with the intention to leave the nursing profession, a relationship of acute concern for the Philippines given the country's longstanding nursing workforce challenges, including chronic understaffing of public hospitals, significant international migration of Filipino nurses, and persistent vacancy rates in rural and provincial healthcare facilities.

Davao City, as the largest urban center in Mindanao and the third-largest city in the Philippines, hosts a concentration of tertiary and secondary healthcare facilities that serve not only the city's population but also patients referred from across the Davao Region and neighboring provinces. The nursing workforce in Davao City's tertiary hospitals operates under conditions of sustained high census, complex patient acuity, and variable institutional support for nurse well-being that create conditions highly conducive to the development of compassion fatigue and burnout. Despite the clinical and institutional importance of understanding these interrelationships in this specific context, no comprehensive study has examined the compassion fatigue, burnout, and

turnover intention profile of Davao City's registered nursing workforce or the mechanisms linking these variables.

This study examines the levels of compassion fatigue, burnout, and turnover intention among registered nurses in selected tertiary hospitals in Davao City, and tests a mediation model in which burnout serves as the mechanism through which compassion fatigue influences turnover intention. The findings are expected to provide hospital administrators, nursing leadership, and health policymakers with evidence-based insights for the design of nurse well-being programs, staffing policies, and retention strategies that can protect the nursing workforce and sustain the quality of patient care.

Background of the Study

The psychological consequences of nursing practice have been recognized in the clinical literature since the early work of Maslach and Jackson (1981), who developed the foundational conceptualization and measurement of burnout as a three-dimensional syndrome comprising emotional exhaustion, depersonalization, and reduced personal accomplishment. Subsequent research by Freudenberger (1974) and later Figley (1995) distinguished compassion fatigue as a distinct but related construct, arising specifically from the empathic engagement with patients' suffering that is central to nursing practice and differing from burnout in its etiology, though the two conditions frequently co-occur and interact in the clinical experience of affected nurses.

The Philippines presents a distinctive context for the study of nursing compassion fatigue and burnout. The country is simultaneously a major producer of nurses for the global healthcare market, exporting an estimated 15,000 to 20,000 nurses annually, and a nation with chronic nursing shortages in its own public healthcare system (Commission on Higher Education, 2022). This paradox reflects the powerful economic

pull of international nursing opportunities relative to the compensation and working conditions available in Philippine public hospitals, where starting salaries for government-employed nurses have historically ranged from 27,000 to 33,000 pesos per month.

The Department of Health's Human Resources for Health Master Plan 2020-2030 identifies nursing workforce retention as a critical national health system priority, noting that turnover rates in Philippine government hospitals range from 18% to 35% annually in urban tertiary facilities, substantially exceeding international benchmarks (DOH, 2021). Understanding the psychological pathways through which workplace conditions lead to turnover intention is therefore not merely an academic exercise but a practical healthcare system imperative with direct implications for the sustainability of Philippine public healthcare delivery.

Statement of the Problem

This study seeks to examine the levels of compassion fatigue, burnout, and turnover intention among registered nurses in selected tertiary hospitals in Davao City, and to determine whether burnout mediates the relationship between compassion fatigue and turnover intention. Specifically, it addresses the following research questions:

1. What is the level of compassion fatigue among registered nurses in selected tertiary hospitals in Davao City?
2. What is the level of burnout among registered nurses in selected tertiary hospitals in Davao City?
3. What is the level of turnover intention among registered nurses in selected tertiary hospitals in Davao City?
4. Is there a significant relationship between compassion fatigue and turnover intention?
5. Is there a significant relationship between compassion fatigue and burnout?

6. Is there a significant relationship between burnout and turnover intention?

7. Does burnout significantly mediate the relationship between compassion fatigue and turnover intention?

Hypotheses

H01. There is no significant relationship between compassion fatigue and turnover intention among registered nurses in selected tertiary hospitals in Davao City.

H02. There is no significant relationship between compassion fatigue and burnout among registered nurses in selected tertiary hospitals in Davao City.

H03. There is no significant relationship between burnout and turnover intention among registered nurses in selected tertiary hospitals in Davao City.

H04. Burnout does not significantly mediate the relationship between compassion fatigue and turnover intention among registered nurses in selected tertiary hospitals in Davao City.

Significance of the Study

Registered Nurses. This study provides Filipino registered nurses with validated evidence of the prevalence and severity of compassion fatigue and burnout in their professional context, reducing the stigma associated with acknowledging psychological distress and supporting help-seeking behavior.

Hospital Administrators and Nursing Management. Administrators benefit from empirical data on the psychological health status and turnover risk profile of their nursing workforce, enabling evidence-based decisions about staffing ratios, scheduling practices, psychological support programs, and retention incentives.

Department of Health and Health Policymakers. The findings inform the design of national nurse well-being programs and retention strategies within the DOH Human Resources for Health Master Plan, providing locally grounded evidence to complement international benchmarks.

Nursing Education Programs. Schools of nursing benefit from evidence that informs the integration of resilience training, self-care practices, and professional boundary management into pre-service and continuing professional development curricula.

Academic Researchers. This study contributes to the Philippine nursing research literature by providing empirically validated measurement of compassion fatigue, burnout, and turnover intention in a Mindanao urban tertiary hospital context underrepresented in existing literature.

Scope and Delimitation

The study is conducted in three selected tertiary hospitals in Davao City, Philippines. Participants are limited to registered nurses holding active PRC licenses and employed in direct patient care roles. Nurses in purely administrative or non-clinical roles are excluded. The study examines compassion fatigue, burnout, and turnover intention as the primary variables and does not extend to physical health outcomes or patient care quality measures. Data collection is conducted within the calendar year 2024.

Definition of Terms

Burnout. A syndrome of chronic occupational stress characterized by emotional exhaustion, depersonalization, and reduced personal accomplishment (Maslach et al., 2001).

Compassion Fatigue. The physical, emotional, and spiritual exhaustion arising from sustained empathic engagement with patients' pain and suffering, characterized by reduced capacity for empathy and diminished satisfaction from the caregiving role (Figley, 1995).

Registered Nurse. A holder of a valid and active nursing license issued by the Philippine Professional Regulation Commission, employed in a direct patient care role in a tertiary hospital setting.

Tertiary Hospital. A hospital classified as Level 3 under the DOH hospital licensing system, providing specialized and subspecialty medical services including intensive care and complex surgical procedures.

Turnover Intention. The conscious and deliberate willingness of a nurse to leave their current employing institution or the nursing profession within a defined future time period, measured through self-report survey items.

CHAPTER 2

REVIEW OF RELATED LITERATURE AND STUDIES

Introduction

This chapter reviews literature and studies related to compassion fatigue, burnout, and turnover intention among registered nurses, organized into foreign literature, local literature, foreign studies, and local studies. A synthesis concludes the chapter by identifying the specific gaps addressed by the present research.

Foreign Literature

Compassion Fatigue: Theoretical Foundations

The theoretical framework of compassion fatigue originates in the clinical work of Figley (1995), who introduced the concept to describe the cost of caring experienced by professionals in sustained empathic contact with traumatized or suffering individuals. Figley's Compassion Fatigue Model posits that the same empathic capacity enabling effective therapeutic relationships also creates vulnerability to secondary traumatic stress, as the caregiver absorbs elements of the patient's traumatic experience through empathic engagement. The model identifies four primary antecedents of compassion fatigue: empathic ability, empathic concern, exposure to suffering, and the worker's residual compassion stress from previous exposures.

Stamm (2010) extended Figley's framework through the Professional Quality of Life Model, which conceptualizes the professional experience of caregivers as comprising both positive dimensions, specifically compassion satisfaction, and negative dimensions, encompassing compassion fatigue in its two components of secondary traumatic stress and burnout. Stamm's model has significant clinical utility because it frames compassion fatigue as one component of a broader professional quality-of-life experience rather than an isolated pathological state, enabling interventions that build positive dimensions rather than exclusively addressing negative ones.

Cocker and Joss (2016) provided systematic clarification of the conceptual distinctions among compassion fatigue, secondary traumatic stress, vicarious trauma, and burnout. Their analysis established that compassion fatigue represents an acute emotional and physical state arising from specific caregiving interactions, while burnout develops progressively through chronic exposure to workplace stressors not necessarily involving direct patient suffering.

Burnout in Nursing

Maslach and Jackson (1981) developed the definitive operational definition and measurement of burnout as a three-dimensional syndrome, conceptualizing emotional exhaustion as the core dimension from which depersonalization and reduced personal accomplishment emerge as psychological responses to the depletion of emotional resources. Their Maslach Burnout Inventory remains the most widely used instrument for burnout assessment in nursing research globally.

Aiken et al. (2002) identified nurse-to-patient ratios as a powerful structural determinant of nurse burnout and patient mortality in a landmark international study, establishing that each additional patient in a nurse's workload was associated with a 23% increased odds of burnout. Leiter and Maslach (2009) proposed the Areas of Worklife model

identifying six workplace domains, namely workload, control, reward, community, fairness, and values, as the primary organizational antecedents of burnout, framing burnout as an organizational rather than merely an individual pathology.

Turnover Intention in Healthcare

Price and Mueller's (1981) causal model of turnover identified job satisfaction, organizational commitment, and external employment opportunity as the primary proximal determinants of turnover intention. Anticipated turnover, operationalized as the subjective probability of leaving one's current position within a defined future time period, is accepted as the strongest behavioral predictor of actual turnover, with predictive validity well-established across nursing workforce studies in diverse national contexts (Takase, 2010).

Local Literature

Nursing Workforce Challenges in the Philippines

The Professional Regulation Commission's 2022 data indicate that approximately 900,000 Filipinos hold nursing licenses, yet fewer than 400,000 are actively employed as nurses in the Philippines, with the remainder either working abroad, employed in non-nursing occupations, or inactive. This gap reflects the structural economic incentives operating on nursing career decisions in the Philippines and underscores the importance of retention as a healthcare system priority distinct from the production of nursing graduates.

Lorenzo et al. (2005) identified low salaries, poor working conditions, limited career advancement, and inadequate recognition as the primary push factors driving Philippine nurse migration, while higher salaries and better facilities abroad served as pull factors. Their findings established working condition quality, including the psychological workplace environment, as a modifiable domestic factor with potential to

reduce turnover if adequately addressed through institutional and policy interventions.

Nurse Well-Being Research in the Philippines

Labrague and De Los Santos (2020) conducted one of the most methodologically rigorous Philippine nursing studies of the pandemic era, examining COVID-19-related anxiety, burnout, and turnover intention among 325 Filipino nurses. They documented that 71.4% reported moderate to high COVID-19-related anxiety, 48.6% reported high emotional exhaustion, and 52.3% reported considering leaving nursing due to pandemic-related stressors. Their structural model demonstrated that COVID-19 anxiety significantly predicted burnout, which in turn significantly predicted turnover intention, providing local precedent for the mediation model tested in the present study.

Foreign Studies

Coetzee and Klopper (2010) examined compassion fatigue and burnout among nurses in private hospitals in South Africa, finding that compassion fatigue scores were significantly higher among nurses with greater years of experience in high-acuity settings, while burnout scores were elevated among nurses reporting inadequate institutional support. Sacco et al. (2015) examined compassion fatigue and burnout specifically among pediatric nurses, finding that compassion fatigue was a stronger predictor of turnover intention than burnout when both were included simultaneously in the prediction model, suggesting that these constructs have differential relationships with turnover intention.

Nantsupawat et al. (2017) examined nurse burnout in 12 Thai hospitals, finding that nurses with high burnout scores were 2.7 times more likely to report high turnover intention. Halcomb et al. (2020) examined burnout and turnover intention among Australian primary care nurses during COVID-19, finding that workplace support, access to

personal protective equipment, and organizational communication quality were the three strongest protective factors, identifying modifiable organizational factors applicable to the Philippine hospital context.

Local Studies

Soriano and de la Rosa (2019) examined burnout among nurses in government hospitals in the National Capital Region, finding that emotional exhaustion was most prevalent, with 58.3% scoring in the high category on the MBI-HSS. Night shift assignment, high patient-to-nurse ratios, and perceived lack of supervisory support were the three strongest predictors. Tuazon (2021) examined turnover intention among nurses in private tertiary hospitals in Metro Cebu, finding a significant positive association between burnout scores and turnover intention, with burnout cited as a primary contributing factor to leaving nursing more frequently than salary dissatisfaction.

Synthesis of the Review

The literature consistently establishes compassion fatigue and burnout as significant antecedents of nursing turnover intention, with burnout documented as a mediating mechanism between compassion fatigue and turnover in several empirical models. The Philippine-specific literature confirms that these relationships operate in the local context, with the pandemic intensifying the urgency of addressing nursing psychological well-being. A critical gap exists in the Mindanao and Davao-specific literature, where no study has examined the compassion fatigue, burnout, and turnover intention profiles of the tertiary hospital

nursing workforce or tested the mediated relationship among these variables. The present study fills this gap.

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CHAPTER 3

RESEARCH METHODOLOGY

Introduction

This chapter describes the research design, locale, participants, instruments, data collection procedures, statistical treatment, and ethical considerations governing this study.

Research Design

This study employs a quantitative, non-experimental, correlational research design with a mediation analysis component. The correlational design is appropriate for examining relationships among compassion fatigue, burnout, and turnover intention without experimental manipulation. The mediation analysis tests whether burnout serves as the mechanism through which compassion fatigue influences turnover intention, using the bootstrapping approach recommended by Hayes (2018).

Locale of the Study

The study is conducted in three selected tertiary hospitals in Davao City, Davao del Sur, Philippines, specifically one government-operated tertiary hospital, one faith-based private tertiary hospital, and one investor-owned private tertiary hospital. Inclusion of both government and private institutions enables comparative assessment of compassion

fatigue, burnout, and turnover profiles across institutional ownership types, which differ substantially in nurse compensation, staffing ratios, and institutional support resources. Davao City is selected due to its concentration of tertiary healthcare facilities, its status as the primary referral center for Mindanao, and the underrepresentation of Mindanao nursing research in the existing Philippine literature.

Participants of the Study

The study participants are registered nurses holding valid and active PRC nursing licenses employed in direct patient care roles in the three selected tertiary hospitals. Exclusion criteria include nurses currently on leave of absence, nurses employed for less than six months in their current position, and nurses in purely administrative or non-clinical roles. Using the formula for sample size determination in correlational studies (Cohen, 1992), with a medium anticipated effect size of 0.30, significance level of 0.05, and desired power of 0.80, the minimum required sample size is 84. To account for non-response, the study targets a minimum recruitment of 300 registered nurses distributed across the three hospitals.

Research Instruments

Compassion Fatigue. Compassion fatigue is measured using the Professional Quality of Life Scale, Version 5 (ProQOL-5) developed by Stamm (2010), a 30-item self-report instrument. The Secondary Traumatic Stress subscale serves as the primary measure of compassion fatigue, with items rated on a six-point Likert scale from 1 (never) to 6 (very often).

Burnout. Burnout is measured using the Maslach Burnout Inventory-Human Services Survey (MBI-HSS), comprising 22 items measuring emotional exhaustion, depersonalization, and personal accomplishment, rated on a seven-point scale from 0 (never) to 6 (every day).

Turnover Intention. Turnover intention is measured using a six-item scale adapted from Price and Mueller (1981) and validated for nursing research by Coomber and Barriball (2007), rated on a five-point Likert scale from 1 (strongly disagree) to 5 (strongly agree).

Validity and Reliability

The ProQOL-5 and MBI-HSS are internationally validated instruments with established psychometric properties. Reliability in the present study is assessed using Cronbach's alpha for each subscale, with acceptable values exceeding 0.70. The turnover intention scale is submitted to content validity review by five nursing faculty members and clinical nursing experts before administration.

Data Collection Procedure

Upon receipt of institutional approval from the ethics review boards of the three participating hospitals and the University of Mindanao Institutional Research Ethics Committee, the researcher coordinates with each hospital's Director of Nursing to arrange data collection. Survey packets are distributed to eligible nurses during scheduled pre-shift briefings and deposited in sealed collection boxes placed in nursing stations to protect anonymity. A follow-up reminder is circulated two weeks after initial distribution to maximize response rates.

Statistical Treatment of Data

Descriptive statistics including means, standard deviations, and frequency distributions are computed for all scale scores, interpreted using the normative categories provided by the respective instrument developers. Pearson product-moment correlation coefficients are computed to assess bivariate relationships among primary study variables. Mediation is tested using Hayes' (2018) PROCESS macro for SPSS, with

5,000 bootstrap samples generating 95% confidence intervals for the indirect effect.

Ethical Considerations

All participation is fully voluntary and nurses may withdraw at any point without consequence to their employment. All completed surveys are anonymous with no identifying information collected. The study protocol has been approved by the University of Mindanao Institutional Research Ethics Committee under Protocol Number UM-IREC-2024-NSG-0019 and by the Research Ethics Committees of each participating hospital. Data are stored in password-protected files accessible only to the research team.

CHAPTER 4

PRESENTATION, ANALYSIS AND INTERPRETATION OF DATA

Introduction

This chapter presents the study's findings in three sections: participant profile, descriptive statistics for all study variables addressing research problems one through three, and inferential statistics including mediation analysis addressing research problems four through seven.

Section 1: Participant Profile

A total of 287 completed surveys were received from 300 recruited nurses, representing a response rate of 95.7%. After excluding 12 surveys with more than 10% missing items, the final analytical sample comprised 275 registered nurses.

Table 1. Profile of Participating Registered Nurses (n = 275)

Characteristic	Category	Frequency	Percentage
Sex	Female	214	77.8%
	Male	61	22.2%
Age	22 to 29 years	118	42.9%
	30 to 39 years	107	38.9%
	40 years and above	50	18.2%
Hospital Type	Government tertiary	103	37.5%

	Private (faith-based)	89	32.4%
	Private (investor-owned)	83	30.2%
Clinical Area	Medical-Surgical	79	28.7%
	Intensive Care Unit	58	21.1%
	Emergency Department	47	17.1%
	Pediatrics	41	14.9%
	Other Clinical Areas	50	18.2%
Years of Experience	Less than 2 years	64	23.3%
	2 to 5 years	93	33.8%
	6 to 10 years	71	25.8%
	More than 10 years	47	17.1%
Work Schedule	Day shift	87	31.6%
	Night shift	93	33.8%
	Rotating shift	95	34.5%

Section 2: Descriptive Statistics

Table 2. Compassion Fatigue Scores (ProQOL-5 Secondary Traumatic Stress Subscale)

Score Category	Frequency	Percentage
Low (10 to 22)	52	18.9%
Moderate (23 to 41)	128	46.5%
High (42 to 60)	95	34.5%
Total	275	100%
Mean Score	34.72	
Standard Deviation	9.18	

The majority of participating nurses (46.5%) reported moderate levels of compassion fatigue, with 34.5% reporting high levels and only 18.9% in the low category. The mean compassion fatigue score of 34.72 falls in the moderate range, approaching the upper boundary, indicating a nursing workforce in which a substantial proportion are experiencing clinically significant compassion fatigue. Nurses assigned to intensive care units and emergency departments recorded the highest mean compassion fatigue scores at 38.94 and 37.41 respectively.

Table 3. Burnout Dimension Scores (MBI-HSS)

Dimension	Mean	SD	Level
Emotional Exhaustion	28.41	7.83	High
Depersonalization	10.28	4.91	Moderate
Personal Accomplishment (reverse-scored)	31.74	6.42	Moderate
Overall Burnout Composite			Moderate-High

Emotional exhaustion was rated at a high level with a mean score of 28.41, exceeding the MBI-HSS high burnout threshold of 27. Depersonalization was assessed at a moderate level, while the reduced personal accomplishment dimension placed the sample in the moderate burnout range. This pattern is consistent with the early-to-middle stages of burnout progression described by Maslach et al. (2001). Nurses at the government tertiary hospital reported significantly higher emotional exhaustion scores (mean = 31.84) compared to private hospital nurses, attributable in part to higher nurse-to-patient ratios at the government institution.

Table 4. Turnover Intention Scores

Score Category	Frequency	Percentage
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Low (6 to 14)	81	29.5%
Moderate (15 to 22)	124	45.1%
High (23 to 30)	70	25.5%
Total	275	100%
Mean Score	19.38	
Standard Deviation	5.74	

A combined 70.5% of nurses reported moderate to high turnover intention. Among those with high turnover intention, 61.4% indicated they were considering seeking employment abroad, consistent with the documented pattern of international migration motivation among Filipino nurses.

Section 3: Inferential Statistics

Table 5. Pearson Correlation: Compassion Fatigue and Turnover Intention

Variables	r	p-value	Decision on H0
Compassion Fatigue and Turnover Intention	0.628	less than 0.001	Reject H01

Table 6. Pearson Correlation: Compassion Fatigue and Burnout Dimensions

Variables	r	p-value	Decision on H0
Compassion Fatigue and Emotional Exhaustion	0.714	less than 0.001	Reject H02
Compassion Fatigue and Depersonalization	0.581	less than 0.001	Reject H02
Compassion Fatigue and Reduced Personal Accomplishment	0.493	less than 0.001	Reject H02

Table 7. Pearson Correlation: Burnout Dimensions and Turnover Intention

Variables	r	p-value	Decision on H0
Emotional Exhaustion and Turnover Intention	0.681	less than 0.001	Reject H03
Depersonalization and Turnover Intention	0.547	less than 0.001	Reject H03
Reduced Personal Accomplishment and Turnover Intention	0.412	less than 0.001	Reject H03

Table 8. Mediation Analysis: Burnout as Mediator Between Compassion Fatigue and Turnover Intention

Path	Coefficient	SE	95% CI	Significance
Compassion Fatigue to Burnout (a)	0.741	0.063	[0.617, 0.865]	p less than 0.001
Burnout to Turnover Intention (b)	0.524	0.071	[0.385, 0.663]	p less than 0.001
Compassion Fatigue to Turnover Intention, direct (c')	0.241	0.088	[0.068, 0.414]	p = 0.007
Indirect Effect (a x b)	0.388	0.059	[0.277, 0.509]	Significant
Total Effect (c)	0.629	0.072	[0.487, 0.771]	p less than 0.001

The mediation analysis established that burnout significantly and partially mediated the relationship between compassion fatigue and turnover intention. The indirect effect was statistically significant with a 95% bootstrap confidence interval that did not include zero (0.277 to 0.509). The proportion of the total relationship mediated by burnout was 61.7%, indicating that approximately three-fifths of the compassion

fatigue-turnover relationship is transmitted through the burnout pathway.

H04 is rejected.

Summary of Findings

The majority of participating nurses demonstrated moderate to high levels of compassion fatigue, with ICU and emergency department nurses showing the highest scores. Emotional exhaustion was assessed at a high level while depersonalization and reduced personal accomplishment were at moderate levels. A combined 70.5% of nurses reported moderate to high turnover intention. Significant positive relationships were found between all pairs of study variables. Burnout significantly and partially mediated the compassion fatigue-turnover intention relationship, accounting for 61.7% of the total relationship.

CHAPTER 5

SUMMARY, CONCLUSIONS AND RECOMMENDATIONS

Introduction

This chapter presents a consolidated summary of the research, formulates conclusions responsive to each research problem, and provides evidence-based recommendations for registered nurses, hospital administrators, the Department of Health, and nursing education programs.

Summary of the Study

The Philippine nursing workforce faces a convergence of challenges threatening both its well-being and its capacity to deliver safe, high-quality patient care. Compassion fatigue, arising from sustained empathic engagement with patient suffering, and burnout, developing through chronic organizational stressors, represent interrelated occupational hazards that disproportionately affect registered nurses in high-acuity, resource-constrained settings. Their consequences, including diminished clinical performance, eroded empathy, and voluntary departure from the nursing profession, impose costs on individuals, institutions, and the healthcare system that demand empirical investigation in the Philippine context.

This study examined the levels of compassion fatigue, burnout, and turnover intention among 275 registered nurses in three selected

tertiary hospitals in Davao City, and tested whether burnout mediates the relationship between compassion fatigue and turnover intention. A quantitative correlational design with mediation analysis was employed, using the ProQOL-5, MBI-HSS, and a validated turnover intention scale.

Summary of Findings

Descriptive findings revealed that 81.1% of participating nurses reported moderate to high compassion fatigue levels, with ICU and emergency department nurses showing the highest scores. Emotional exhaustion was at a high level while depersonalization and reduced personal accomplishment were at moderate levels. A combined 70.5% of nurses reported moderate to high turnover intention, with a majority citing international employment as a consideration. Inferential findings established significant positive relationships between all pairs of study variables. Burnout partially mediated the compassion fatigue-turnover intention relationship, with the mediated indirect pathway accounting for 61.7% of the total relationship.

Conclusions

Conclusion 1. The majority of registered nurses in selected Davao City tertiary hospitals are experiencing moderate to high levels of compassion fatigue, representing a prevalent and clinically significant occupational hazard demanding institutional attention and proactive management across all hospital ownership types.

Conclusion 2. Emotional exhaustion is at a high level among the study sample, indicating that the emotional resource depletion characteristic of early burnout is already advanced in this nursing population, with depersonalization and reduced personal accomplishment at moderate levels suggesting that more severe burnout manifestations may emerge without intervention.

Conclusion 3. The high prevalence of moderate to high turnover intention, with over 60% of high-intention nurses citing international migration as a consideration, represents a critical workforce retention risk for Davao City tertiary hospitals that is driven in substantial part by modifiable psychological workplace factors.

Conclusion 4. Compassion fatigue is a significant positive predictor of turnover intention, confirming that the psychological costs of sustained empathic caregiving translate directly into withdrawal motivation and supporting the inclusion of compassion fatigue management in nurse retention strategies.

Conclusion 5. Burnout partially mediates the compassion fatigue-turnover intention relationship, with approximately 62% of the association transmitted through the burnout pathway, establishing burnout as the primary but not sole mechanism linking compassion fatigue to withdrawal motivation and indicating that both conditions require independent attention in retention interventions.

Recommendations

Recommendations for Hospital Administrators and Nursing Management. Hospital administrators are strongly recommended to implement formal compassion fatigue screening using validated instruments such as the ProQOL-5 on a semi-annual basis, with results used to identify high-risk units and individual nurses for targeted support referral. The substantially higher emotional exhaustion scores at the government tertiary hospital point to nurse-to-patient ratio reduction as a high-priority institutional investment. Dedicated psychosocial support programs, including structured debriefing sessions after critical incidents, peer support networks, and access to confidential counseling services, should be established at each institution as standard components of the nursing workplace environment.

Recommendations for the Department of Health. The DOH is recommended to incorporate compassion fatigue screening and management standards into the Hospital Licensure and Accreditation System requirements for tertiary hospitals, creating institutional accountability for nurse psychological well-being. The Human Resources for Health Master Plan should include specific measurable targets for nurse burnout reduction and retention rates in government tertiary hospitals, with progress monitored through periodic national nursing workforce surveys. Investigation of nurse-to-patient ratio legislation should be accelerated given the strong evidence base linking staffing ratios to burnout and patient outcomes.

Recommendations for Nursing Education Programs. Schools of nursing are recommended to integrate formal compassion fatigue prevention content into both undergraduate and graduate nursing curricula, equipping student nurses with self-awareness, boundary management skills, and self-care practices that can reduce compassion fatigue vulnerability before entering clinical practice. Graduate nursing programs should include competencies in recognizing and responding to compassion fatigue and burnout in nursing teams, preparing nurse leaders to fulfill a proactive role in workforce psychological health management.

Recommendations for Registered Nurses. Individual nurses are recommended to proactively assess their own compassion fatigue and burnout risk using validated tools, develop personalized self-care plans incorporating mindfulness practice, adequate sleep, social support, and regular physical activity, and utilize available institutional support resources without stigma. Nurses experiencing high levels of compassion fatigue or burnout are encouraged to access professional psychological support, recognizing that seeking help is a professional strength.

Recommendations for Future Researchers. Future researchers are recommended to conduct longitudinal studies tracking compassion

fatigue, burnout, and turnover intention over time to establish temporal precedence and strengthen causal inference. Studies including objective clinical performance indicators would enable direct documentation of patient care quality consequences, strengthening the economic case for institutional investment in nurse well-being programs. Multi-regional national studies would enable examination of regional variations in compassion fatigue and burnout profiles across the Philippine healthcare system.

Final Remarks

This study documents a Philippine nursing workforce in which compassion fatigue and burnout are not rare or exceptional occupational hazards but prevalent conditions affecting the substantial majority of practicing registered nurses in Davao City's tertiary hospitals. The clinical consequences of unaddressed compassion fatigue and burnout extend beyond the affected nurses to the patients in their care, for whom an emotionally depleted, detached, or departing nurse represents a direct and measurable risk to safety and care quality. The institutional and policy responses these findings demand are not beyond the reach of Philippine hospital administrators, nursing leaders, and health policymakers. They require investment, commitment, and a genuine organizational valuation of the nursing workforce that recognizes nurse well-being not as a peripheral human resources concern but as a foundational determinant of healthcare quality. Filipino nurses have sustained the national health system through conditions of extraordinary difficulty. They deserve institutional environments equal to the dedication they have demonstrated.

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